

U.S. Healthcare Services

US Healthcare Services: A summary of our teach in on Medicare Advantage



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We kicked off our Summer Healthcare Teach In Series last Friday, providing an overview of Medicare Advantage, its history, policy and margin impacts, and future outlook ([replay here](#) and [slides here](#)). This note provides key takeaways from our session.

We will continue our Summer Teach In series with a session on Optum Health (6/18) at 11am ET ([Click here to register](#));

Insurance cycle - We see the recent negative margins in MA as having been the result of a traditional insurance cycle driven by excessive competition. Revenue growth rates in the 15-20% range during 2005-2015 and margins of around 5% attracted a significant amount of additional competition. This competition led to reduced margins and set the stage for a margin crisis as rates shocks occurred.

We see MA margins inflecting after a trough period. Following a period of margin compression, we expect industry MA margins to expand ~1% in 2026, followed by ~0.5% annual growth through 2030. This recovery is driven by a withdrawal of competition, rate improvement off 2024–25 lows, and an industry focus on pricing for profitability.

MA growth remains durable, but is moderating. Moving forward, we expect MA enrollment to grow at a ~4–5% pace. The value proposition remains compelling with MA offering enhanced supplemental benefits for beneficiaries and structurally lower costs versus FFS for the government. We ultimately see MA capturing virtually all the Medicare market.

The long-term Government MCO market will trend “utility-like.” Over time, we expect both MA and Medicaid to resemble utility-like markets, consolidating around ~4–5 scaled players and operating with relatively modest margins (~1.5% for Medicaid; ~2–3% for MA).

Key disruptors include GLP-1s, AI, and VBC. GLP-1s will contribute to rising pharmacy costs, but offer the potential of increasing life span, potentially expanding Medicare TAM by 5-10%. AI offers the opportunity to improve the efficiency of hospital operations and patient care. VBC will become increasingly central as margin pressure persists, driven by our view that reimbursement rates will lag medical cost trend by ~100–150 bps long-term.

Affordability and access will dominate the policy debate. Policy focus is increasingly centered on affordability and coverage expansion. Medicare for All appears less viable than in prior cycles, particularly with MA now representing ~55% of Medicare enrollment vs. ~1/3 during earlier reform debates. We expect a Democratic push to restore the uninsured rate to around the 5% level, from what we see could drift towards a potential 9% by 2028.

BERNSTEIN TICKER TABLE

Ticker	Rating	8 Jun 2026		Price Target	TTM Rel. Perf.	Adjusted EPS				Adjusted P/E (x)		
		Cur	Closing Price			Cur	2025A	2026E	2027E	2025A	2026E	2027E
AGL (agilon health)	M	USD	95.33	86.00	53.1%	USD	(24.46)	(4.81)	(4.52)	(3.9)	(19.8)	(21.1)
CNC (Centene)	O	USD	65.03	68.00	(5.5)%	USD	2.08	3.38	4.23	31.2	19.2	15.4
CI (Cigna)	O	USD	289.61	371.00	(30.7)%	USD	29.84	30.38	33.00	9.7	9.5	8.8
CVS (CVS)	O	USD	97.08	106.00	29.3%	USD	6.75	7.37	8.14	14.4	13.2	11.9
ELV (Elevance)	O	USD	418.15	424.00	(17.1)%	USD	30.30	26.93	28.88	13.8	15.5	14.5
HCA (HCA)	M	USD	361.32	413.00	(29.4)%	USD	28.21	30.44	33.14	12.8	11.9	10.9
HUM (Humana)	O	USD	355.98	425.00	30.9%	USD	16.60	9.12	15.41	21.4	39.0	23.1
MOH (Molina)	O	USD	198.41	208.00	(56.6)%	USD	11.03	5.35	9.65	18.0	37.1	20.6
UNH (UNH)	O	USD	406.57	492.00	10.7%	USD	16.35	18.86	21.99	24.9	21.6	18.5
SPX			7,405.73									

O - Outperform, M - Market-Perform, U - Underperform, NR - Not Rated, CS - Coverage Suspended

Source: Bloomberg, Bernstein estimates and analysis.

INVESTMENT IMPLICATIONS

We rate AGL Market-Perform with a target price of \$86, CNC Outperform with a target price of \$68, CI Outperform with a target price of \$371, CVS Outperform with a target price of \$106, ELV Outperform with a target price of \$424, HCA Market-Perform with a target price of \$413, HUM Outperform with a target price of \$425, and UNH Outperform with a target price of \$492.

DETAILS

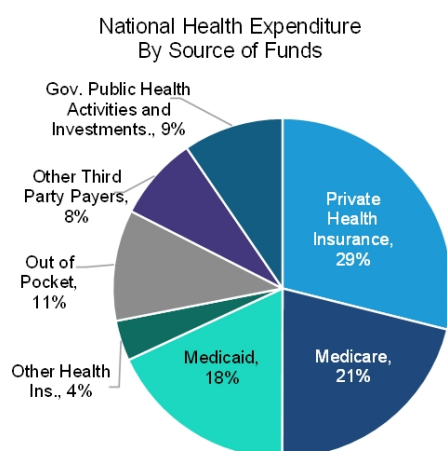
WHAT IS MEDICARE ADVANTAGE?

Medicare covers about 20% of the population. Medicaid and safety net programs cover ~30% and employer ~50%. However, the amount of spend on Medicare is larger than its proportion of members due to the increased medical spend by the senior population.

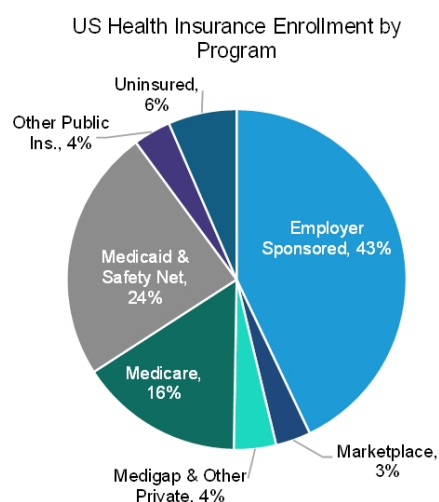
EXHIBIT 1: Where Medicare fits into health insurance coverage

Major US Insurance Programs – Spends and Enrollments

US Health Insurance - Spends



US Health Insurance - Enrollment



Source: CMS, NHE, and Bernstein analysis

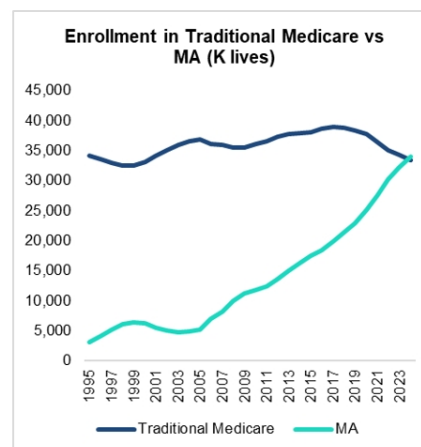
Medicare has ~70m enrollees, with 90% being 65 years old or older and the remainder being disabled. The Medicare population is a growing population, with an age-in growth rate around 5.5% and a mortality rate around 4.5%. Increasing life expectancy, potentially influenced by GLP-1s, is poised to be a tailwind for Medicare member growth. Virtually all the growth in the Medicare population since the 1990s has been in Medicare Advantage, with traditional Medicare staying flat.

EXHIBIT 2: **MA market size**

Medicare is around 70m enrollees with 90% 65 years old and older

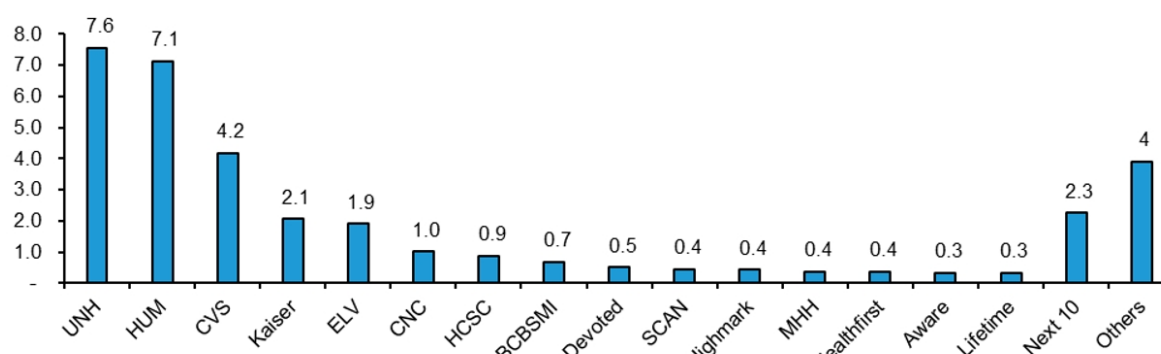
- Medicare enrollment – 70 m
 - Aged 90%; Disabled 10%
 - MA 51%; FFS 49%
 - Age bands: 65-74, 52%, 75-84 35%, 85+ 12%
- Average mortality rate of 4.5 %
- Age-in growth rate - around 5.5%
- Life expectancy gains
 - Historically +1.8yrs/decade
 - Recent US experience +0.6 yr/decade
 - GLP1s could add an additional +1 yr/decade

Most of the growth in Medicare has been in MA over the past decades



Source: CMS, Bernstein analysis

UNH is the largest player in the MA space. UNH and HUM have been the #1 and #2 players for a decade, with CVS being a consistent #3. There is a long tail of other competitors in the space, with the big 3 players representing ~55% of the market. It is a relatively fragmented market, but our long-term view is the industry becoming utility-like, with 4-6 large, scaled players.

EXHIBIT 3: **Who are the major players in Medicare Advantage****MA Membership of major MCOs**

Next 10 includes: Alignment, Corewell, Medica, Guidwell Mutual, UPMC, MOH, BCBSTN, CuraCor, Clover, Independence Health

Source: CMS, Company reports, Bernstein analysis

THE MA PRODUCT, DISTRIBUTION, AND VALUE PROPOSITION

Value proposition of MA. In general, traditional medicare pays for 80% of healthcare costs and the patient pays for 20%. Members have to purchase stand-alone drug coverage and supplemental insurance to pay for the remainder. MA plans cover

these extra expenses, usually have out of pocket maximums, and provide supplemental benefits like vision, dental, and hearing aids.

EXHIBIT 4: Traditional Medicare vs MA

Traditional Medicare

Medical coverage

Part A – hospital coverage
Part B – physician coverage

Price / structure

- \$0 premium for Part A; monthly premium for Part B
- 80% Medicare / 20% patient

Medicare Supplement

- Seniors can buy Medicare Supplement insurance to cover their cost share and cap financial exposure

Drugs

Seniors can purchase Part D insurance

Medicare Advantage

Medical coverage

Part A – hospital coverage
Part B – physician coverage

Drugs

Included in the MA plan (MAPD)

Other benefits

- Vision, dental, hearing
- OTC items allowance

Price/Structure

- Members are typically limited to in-network providers
- The most popular format is \$0 premium plans
- Co-pays or coinsurance for procedures
- Annual maximum out of pocket limit of \$8,850 for medical services

Source: Bernstein analysis

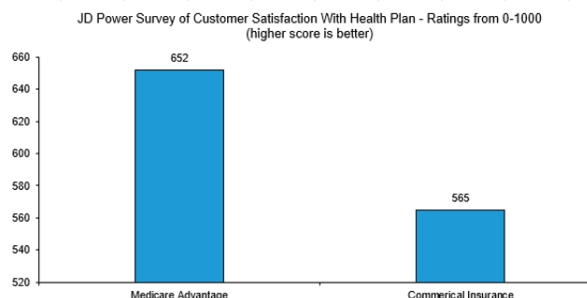
Financial elements are generally the highest priority buying criteria for consumers. 65-70% of the MA market has zero premiums, and the remainder generally has a small premium. A second concern for consumers is whether their preferred provider is in network, which can be a reason to avoid MA for some seniors. Supplemental benefits such as cash cards, vision, and chiropractic are also important determinants of consumer satisfaction. Brand and ratings is of lower importance, but Star ratings can be a major determinant impacting the deductible or supplemental benefits a plan provides. In general, consumers are happier with MA than they are with commercial insurance. This could be due to the lower monthly premiums and deductible associated with MA. Consumer satisfaction surveys between MA and FFS Medicare reveal comparable satisfaction ratings, with prior authorizations and network restrictions being the main consumer complaints surrounding MA.

EXHIBIT 5: MA plan buying criteria

Buying Criteria

- Premium
- Out of pocket expenses
- Is my doctor and drug in network? (network and formulary)
- Coverage details
- Supplemental benefits
- Brand and ratings

Satisfaction surveys



Source: JD Power Survey; Commonwealth Fund survey; Bernstein analysis

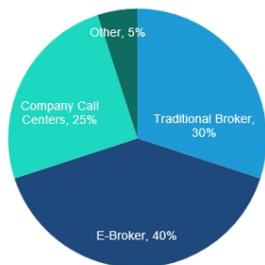
MA is sold most frequently through a broker. We estimate ~40% of sales are through E-Brokers and ~30% are through traditional brokers, with a subset of the broker market being the MCOs' internal brokerage, and most of the remaining sales

being through MCO call centers. Customer segments are made up of individual (~60%), group (~20%), and dual Medicare/Medicaid (~20%). Members of Group MA have access to retirement benefits through a previous employer. The differentiation between customer segments becomes more important as we think about the winners and losers in the MCO market on a forward basis. For example, price sensitive shoppers could be interested in programs offering cash cards while other customers may be solely focused on maintaining their physician network.

EXHIBIT 6: MA go-to-market

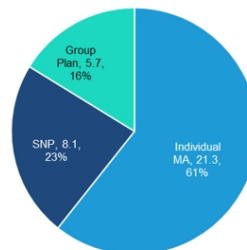
Distribution channels typically include a broker

Estimated MA Plan Sales Distribution



Customer segments

- Individual, Group, Duals



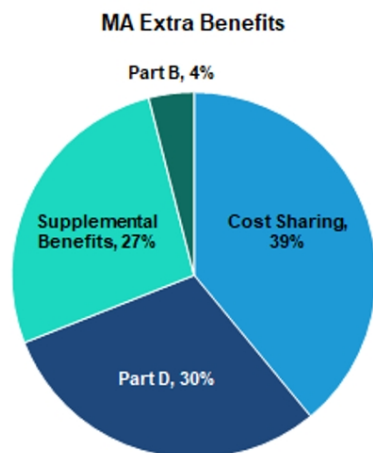
- Customer segments
 - Price sensitive shoppers
 - Network loyalists
 - Benefits maximizers
 - Chronic condition members
 - Brand trusting
 - Convenience

Source: Bernstein estimates and analysis

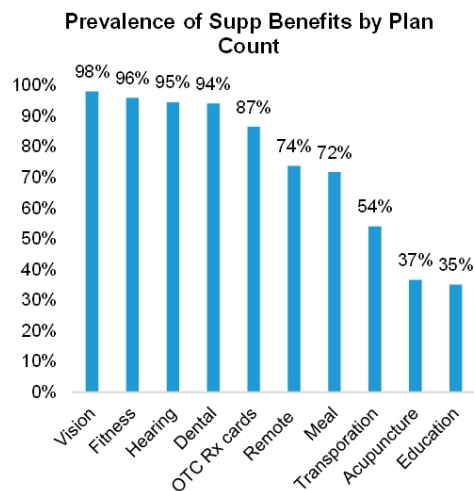
The reason patients switch to MA is typically the extra benefits it provides. Extra benefits include Silver Sneakers, hearing aids, and other benefits which appeal to seniors. Every year, members have to reaffirm their choice between MA and FFS Medicare, providing an opportunity for additional members to enter MA. On average, MA members receive \$194 per month in extra benefits, equating to ~15% extra value. The largest proportion of extra benefit is cost sharing, which includes lower copay and plan maximums. The second-largest component is the included Part D coverage, and the third largest is supplemental benefits. Around 2017-2019, MA plans began offering benefits to cover part or all of Part B, and this has become the fastest growing component of extra benefits. The extra benefits that are offered can be another differentiating feature in product design.

EXHIBIT 7: **Extra benefits for seniors in MA**

In 2024, MA members received on average \$194 per month in extra benefits



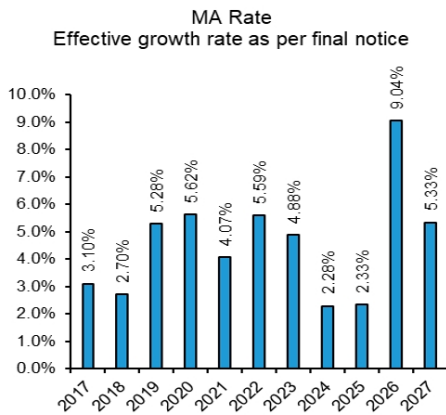
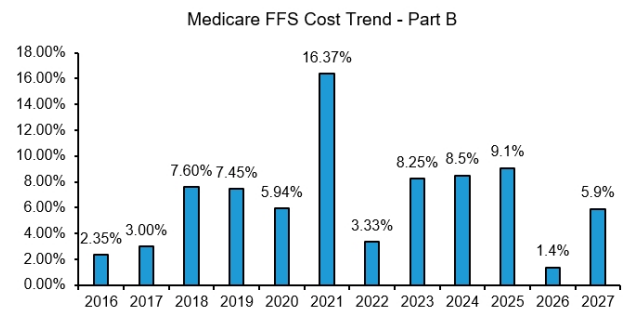
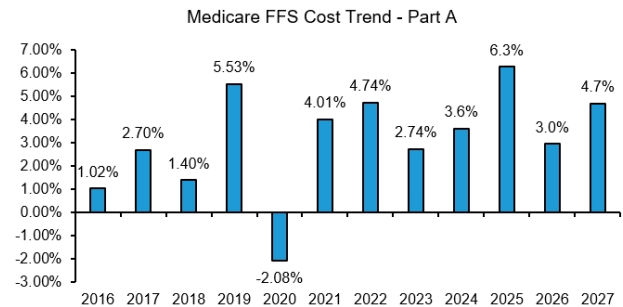
The supplemental benefits are additional benefits not found in Traditional Medicare



Source: CMS and Bernstein analysis

MEDICAL COSTS AND MA RATES

Cost trend has been growing faster than MA rate increases, which was a major reason for MA underperformance in 2023, 2024, and 2025. CMS declares each year what the growth rate has been for medical costs, which has been around 4-5%. Medicare Part A (hospital) costs trended around 2.5-3% pre-COVID and have been higher post-COVID due to wage inflation and increased utilization. Part B (outpatient) has run higher than Part A. When combining inpatient and outpatient expenses, cost trend has averaged around 4-5%, as can be seen in Exhibit 8.

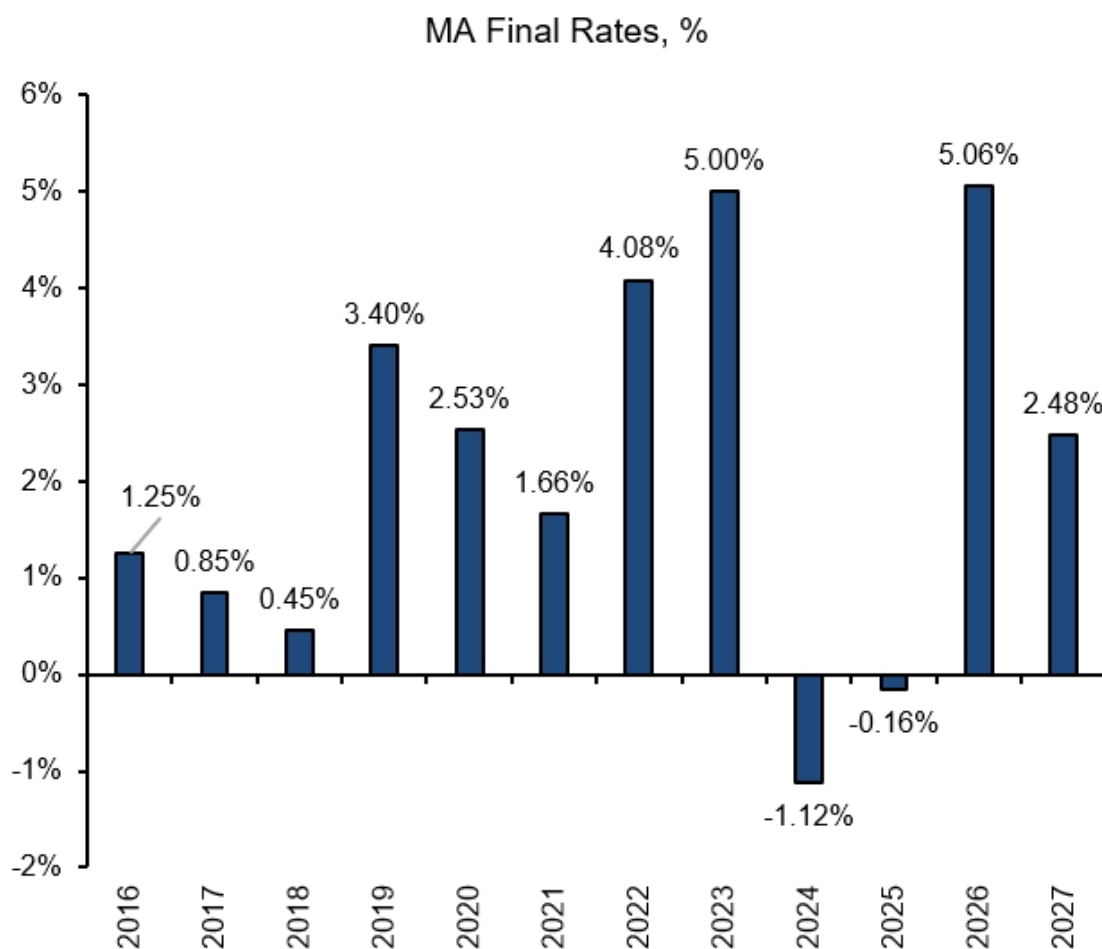
EXHIBIT 8: **MA medical cost trend****MA effective growth rate – this should represent trend****During 24/25 we see effective growth rate as undercounting medical cost trend.**

Source: CMS and Bernstein analysis

MA rates vary year to year, typically driven by phases of underpayment or catch-up on rates. Over the long term, MA rates have tended to range around 2.5%, while trend has averaged around 4.5%, so rates have been around 2 points below trend. Rates have tended to be lower during Democratic administrations and higher during Republican administrations (the rates typically are lagged by 3-4 years to the administration).

EXHIBIT 9: MA rates

MA Final rates



Source: CMS, Bernstein analysis

MA rate buildup. CMS typically puts out an advance rate notice around February 1st and a final notice around April 1st for the next year. The effective growth rate is the medical cost trend, which is on the top row in [Exhibit 10](#). Then, there's a series of adjustments including rebasing, risk model revision (v28), and Star ratings. The MA rate is typically presented without the risk score. The risk score is additive; it's revenue any MCO receives due to having a riskier population. Theoretically, the extra costs an MCO incurs from having a risky patient population should be offset by the risk score.

EXHIBIT 10: **MA rate notices****2023 Notice**

Year-to-Year Percentage Change in Payment

Impact	2023 Advance Notice	2023 Rate Announcement
Effective Growth Rate	4.75%	4.88%
Rebasing/Re-pricing	N/A ^[1]	0.39%
Change in Star Ratings	0.54%	0.54%
Medicare Advantage Coding Pattern Adjustment	0%	0%
Risk Model Revision	0%	0%
Normalization	-0.81%	-0.81%
MA risk score trend ^[2]	3.50%	3.50%
Expected Average Change in Revenue	7.98 %	8.50%

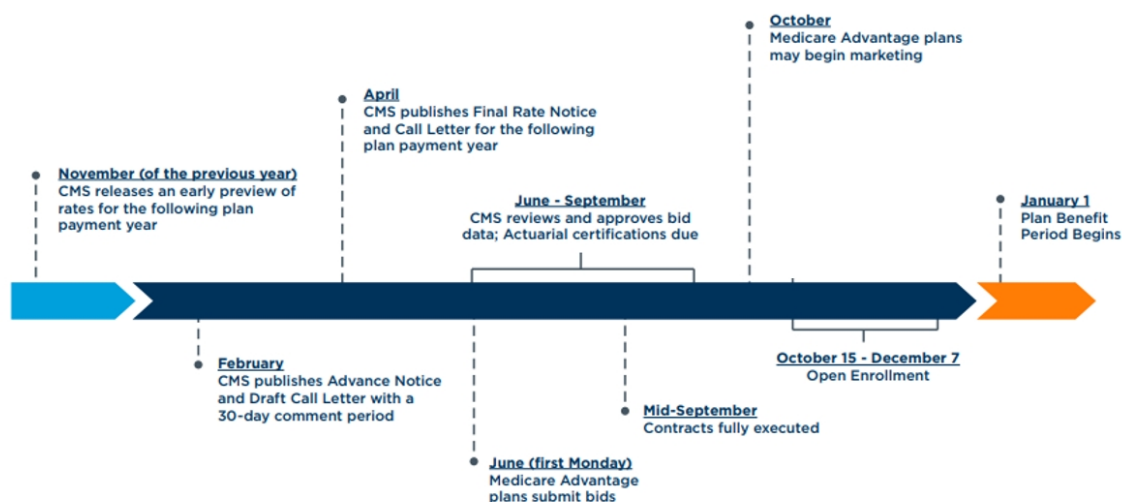
2024 Notice

Year-to-Year Percentage Change in Payment

Impact	2024 Advance Notice	2024 Rate Announcement
Effective Growth Rate	2.09%	2.28%
Rebasing/Re-pricing	N/A ^[1]	0.00%
Change in Star Ratings ²	-1.24%	-1.24%
Medicare Advantage Coding Pattern Adjustment	0%	0%
Risk Model Revision and Normalization ³	-3.12%	-2.16%
MA risk score trend ⁴	3.30%	4.44%
Expected Average Change in Revenue	1.03 %	3.32%

Source: CMS, Bernstein analysis

Rate notices and cost trend evaluation are done in the year before plans take effect. In February, the initial rate increases are released, and the final rate is released in April. The rates can vary based on geography. MCOs observe trend throughout the year, and MA plan bids are submitted in early June. When an MCO is developing their plan, they consider the rate increase, cost trend, and desired product features before submitting their bid. Plans are finalized in October and open enrollment begins October 15th for members to choose their plan. The main factors to monitor during the plan year is whether negative selection bias has been introduced by changes to plans and whether there have been changes in utilization. Q2 results are often a good barometer of whether MCOs will observe impacts from benefit design.

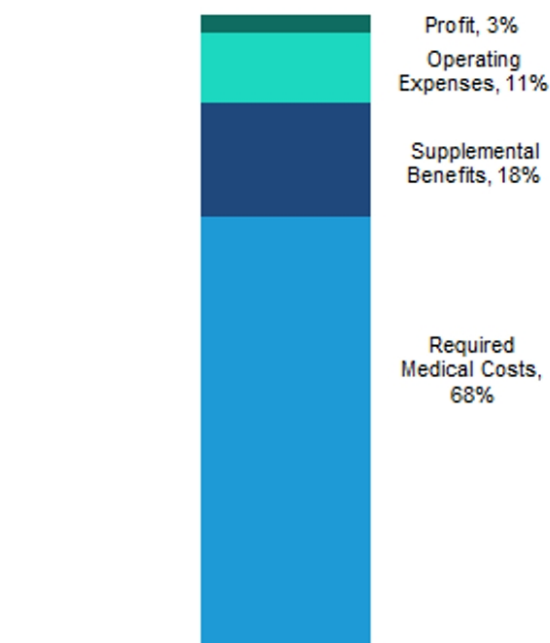
EXHIBIT 11: **MA annual bid timeline**

Source: CMS, Bernstein analysis

MCO bid construction. The formula for constructing a bid includes estimating medical costs, determining extra benefits and premium levels, and determining operating costs to define a target profit level. It's generally perceived that MA saves about 30% compared to Medicare FFS. In MA, the extra savings are used for supplemental benefits and operating expenses. Coming into 2026, the advance notice rate increase was ~0%. A potential side effect of this margin compression could have been "shrinkflation." This is seen in consumer goods where manufacturers shrink package size. A similar mechanism can occur in the MA industry by decreasing benefits and increasing the deductible. MCOs can also improve margins through things like VBC to improve costs. This mechanism is core to our thesis that VBC's value proposition will increase in the face of prolonged margin compression. Exiting unprofitable markets is another option when faced with margin pressure, which can be a quick way to achieve margin expansion.

EXHIBIT 12: **How an MCO constructs a MA bid**

-
- + Estimate required medical costs
 - + Determine expected premium level
 - + Forecast competitor offerings of supplemental benefits
 - + Determine operating costs and targeted profit level

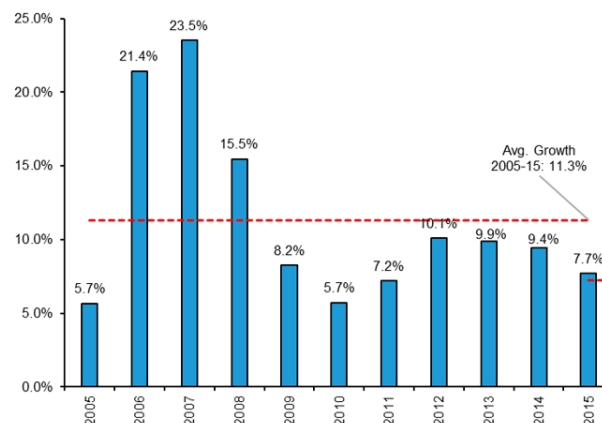
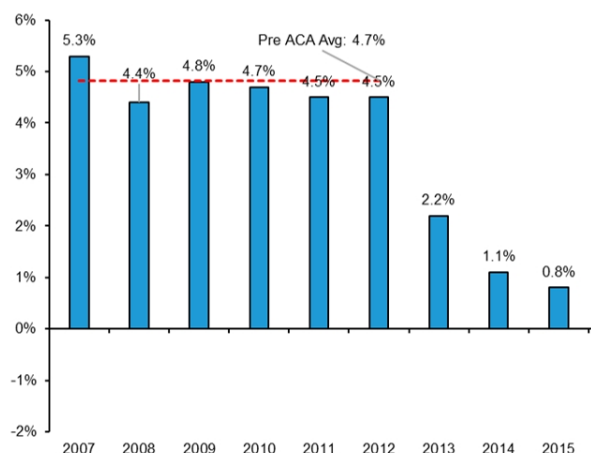


Source: Bernstein analysis

There are several levers to improve MA revenue. In traditional businesses like Employer, pricing discipline involves increasing rates, but in MA the government largely sets the rate increases. However, MA plans still have several mechanisms to impact revenue. The first is Star ratings, which provide bonuses for quality, and the second is risk adjustment, which provides increased payments for more complex patients. MCOs receive 5% extra payment for members who are in 4-star plans or above. These typically flow out as extra benefits but can also be a lever to improve margins. Additionally, MA plans can influence the cost of their product by exiting certain markets or products, cutting benefits, increasing deductibles, steering patients to lower cost providers through network management, and reducing operating costs.

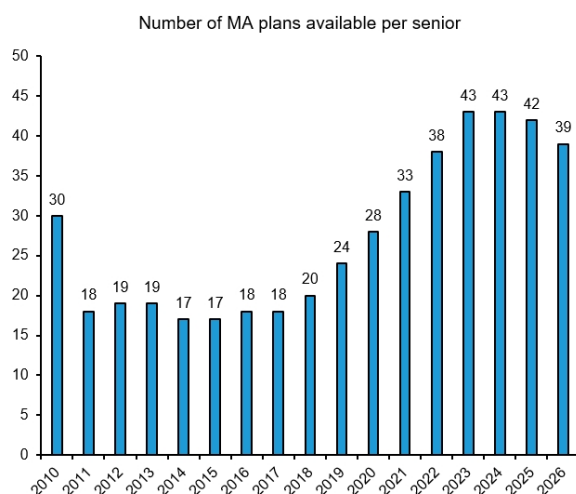
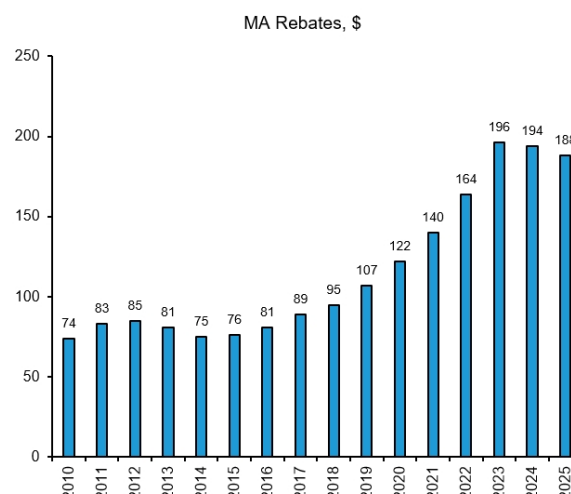
WHAT DROVE THE RECENT CRISIS IN MA PERFORMANCE

We believe the seeds of the current Medicare crisis grew from excessive competition which was attracted by growth. MA membership grew 11+%/year in the years from 2005-2015. This strong growth and 5% margin attracted many competitors to the space.

EXHIBIT 13: **MA membership growth****MA had very strong growth in early years****With initial margins nearly 5%, although impacted by the ACA rate shock in 2013-2015**

Source: CMS, Medicare Trustees Report, Bernstein analysis

The number of MA plans per senior doubled from 2011 to 2023. We've estimated 3 companies were competing for each senior in the late 2000s, increasing to 6 companies per senior in 2018 and 9 per senior in 2023. As competition increased, so did the amount of extra benefits offered. The dollar value of extra benefits more than doubled from the mid 2010s to the mid 2020s, putting stress on MA margins. This caused traditional Medicare players to increase their focus on growth, while non-traditional players such as employer and Medicaid MCOs expanded into MA to gain access to this growth.

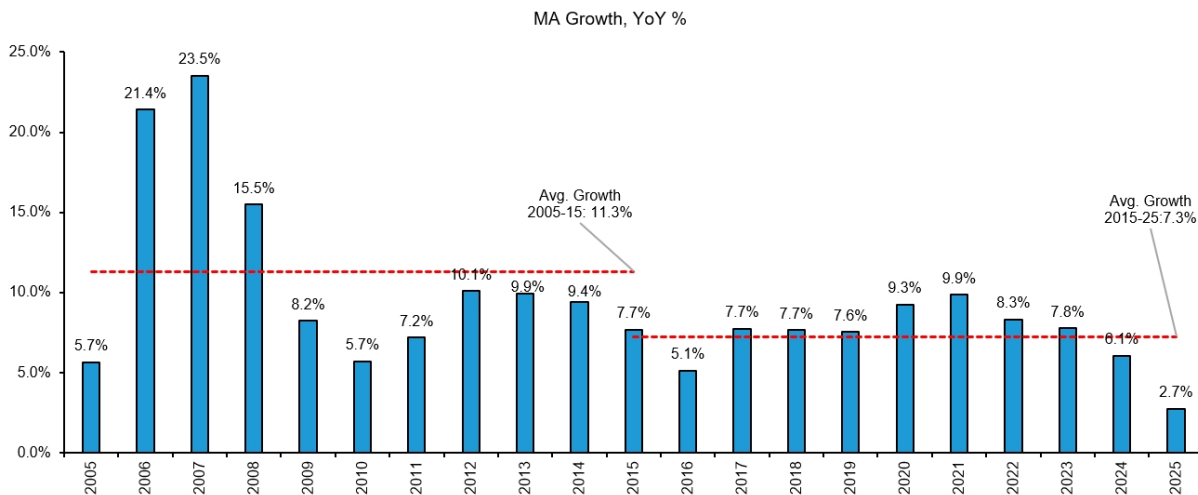
EXHIBIT 14: **Medicare operating margin****This growth and margin profile attracted competitors****And plans increased extra benefits for seniors to grow share**

Source: KFF, Bernstein

MA membership growth slowed as the sector matured. As the sector matured, membership declined from the 11+%/growth seen in 2005-2015 to 7% growth in 2015-2025. While still an attractive sector, revenue growth began slowing.

EXHIBIT 15: MA membership growth

MA membership continued to grow, but at a slower % rate given its larger base

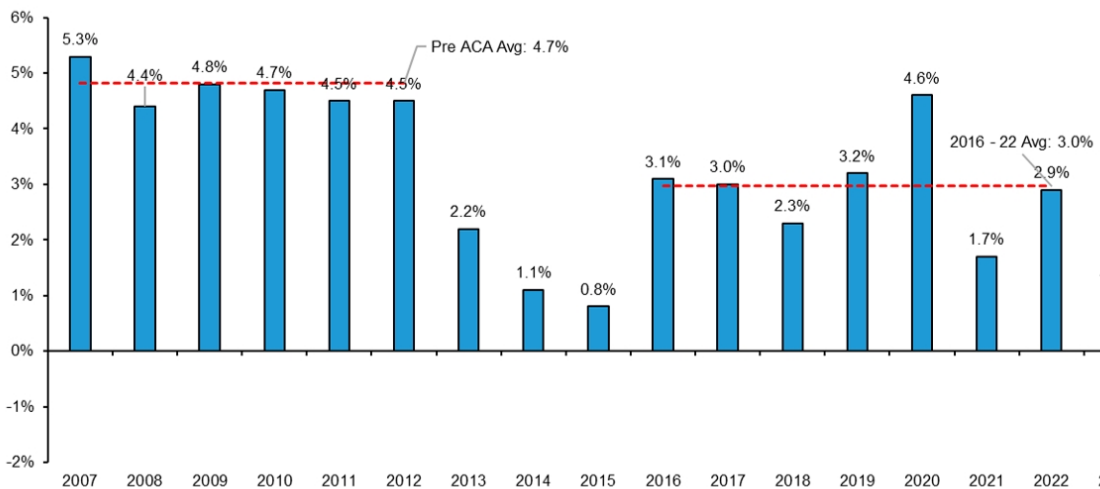


Source: CMS, Medicare Trustees Report, Bernstein analysis

The increased competition resulted in margin compression, from close to 5% operating margins in 2007-12 to 3% in 2016-2022. Even before the recent crisis hit in 2023, competition was already having an impact on MCO margins. A rate shock can be observed from 2013-15 in [Exhibit 16](#) below. Every ~12–15 years the industry sees a rate shock where the federal government cuts MA funding. This was seen in 1998 under President Clinton and 2010 under President Obama. Typically, these changes are legislated, so there is time for the market to prepare for the policy. Under Clinton, the impact of the legislation was felt in 2000-2002, and under Obama the impact was felt in 2013-2015. Historically, MCOs have reacted slowly and have experienced margin compression for several years before margins stabilized at a lower level.

EXHIBIT 16: **Medicare operating margin**

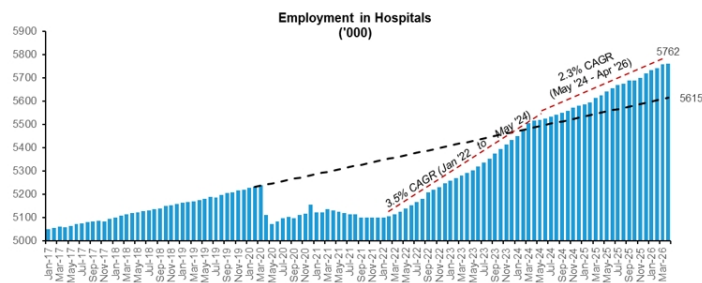
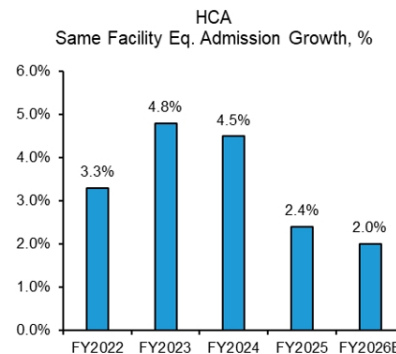
And margins compressed from around 5% to around 3%



Source: NAIC, Bernstein analysis

THE RECENT CRISIS

In 2023, we saw the remnants of a supply shock associated with COVID. Over time, growth in hospital employment closely mirrors growth in same facility admissions. The 3rd and 4th quarters of 2020 experienced a supply shock recession, and employment in the healthcare sector was slower to bounce back than other industries. Hospital employment began to rebound in 2022, with hospital employment growth rate increasing 1-1.5 points faster than historical norms leading to a strong increase in hospital volumes. The significant increase in utilization impacted MCO margins in 2023. In recent years, hospital employment has overshot its historical trendline leading to a decline in hiring and a subsequent normalization in utilization, creating a tailwind for MCOs in 2025 and seemingly in 2026.

EXHIBIT 17: **Utilization cycle was the big impact for 2023****Supply shock impacts capacity****And catch up in capacity growth surprises MCOs in 2023/2024**

Source: BLS, HCA, Bernstein analysis and estimates

Every 12 - 15 years we have experienced Medicare spending shocks over the past few decades - with the 1997 BBA under Clinton and the Obama ACA bill being the most recent legislative pushes for this. Generally these rate shocks have lasted 2–3 years before the pain on the seniors through benefit cuts or reduced provider supply leads to normalization of rates. The swiftness of the Biden executive actions was unusual compared to previous policy impacts. These were executive actions, not legislative actions, so they occurred quicker than the previous Congressional bills. The Biden executive actions were enacted in 2022-24 and began having impacts in 2024-25.

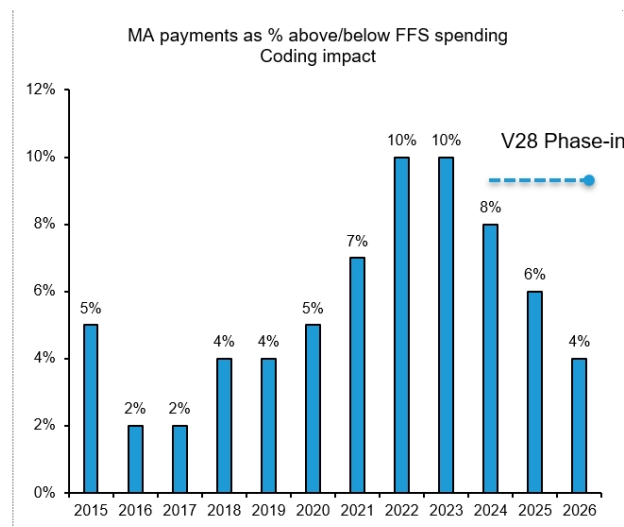
EXHIBIT 18: **History of Medicare / MA reform****Major legislation impacting Medicare and Medicare Advantage**

<u>Year</u>	<u>Bill</u>	<u>Impact</u>	<u>Senate vote</u>
1965		Medicare established	68-21
1982	TEFRA	Medicare Risk program	52-47
1997	BBA	Medicare + Choice	85-15
2003	MMA	Medicare Advantage, Part D	54-44
2010	ACA	MA Reforms	60-39
2022-24		Executive actions	

Source: Bernstein analysis

The Biden executive actions were driven by a concern around excess coding. There was a perception that excess coding drove 10% overpayments in 2022-23, so v28 was phased in to reduce risk adjustments by ~6% over 3 years. The 2027 rate reduces this excess coding by an additional 1.5%.

EXHIBIT 19: Risk adjustment

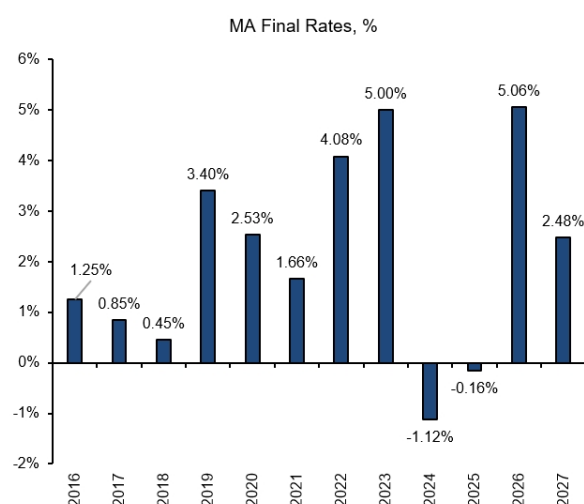
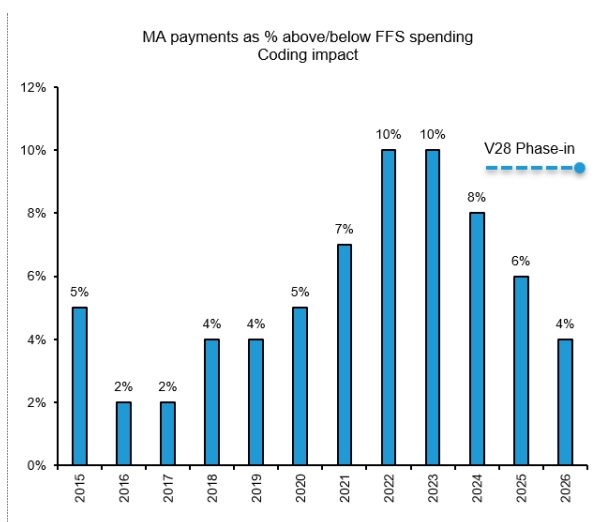
**Risk adjustment accounted for 10% overpayments in 2022-23
V28 phase-in reduced risk adj. impact to 4% in 2026****How does MA Risk Adjustment works and v28 changes?**

- Basic premise is MA plans should be paid more to manage members that have costlier conditions.
- CMS introduced v28 in 2023, to be phased in over a three-year period.
- Constraining process - CMS also decided not include HCCs in the new model where conditions did not accurately predict costs
- The net impact of these changes has been a 2-3% annual headwind to premiums (reflected in lower MA rates) since 2023 (CY26 is the last year of transition).

Source: MedPAC, Bernstein analysis

This concern over excess coding led to low MA rates during 2024/2025. The low effective rates were an executive agency effort to cut rates, and were coupled with the risk adjustment formula changes in v28.

EXHIBIT 20: MA rates

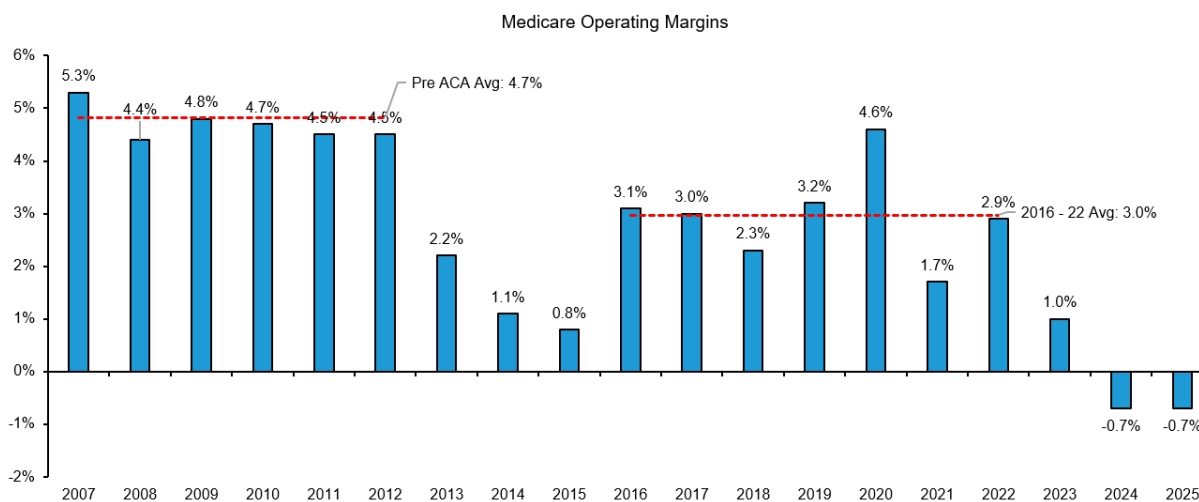
MA Final rates**V28 to address overcoding**

Source: CMS, MedPac, Bernstein estimates and analysis

Industry margins have declined 360 bps since 2022. Margins have compressed over the past three years, with a very competitive market confronting a surprise post-COVID utilization spike in 2023, and the low rates of 24/25 and risk adjustment policy changes further reducing margins. We saw a decline of 190 bps in 2023 - this was predominantly due to mis-pricing

resulting from the post-COVID utilization uptake. We note that pricing discipline and withdrawal of competition beginning in 2025 allowed for margin stability in spite of a low rate and v28 implementation. We expect improved industry margins in 2026 and expect 2027 MA margins will increase although at a slower rate than the 26 improvement given commentary on 2027 pricing discipline by MCOs.

EXHIBIT 21: Medicare operating margin



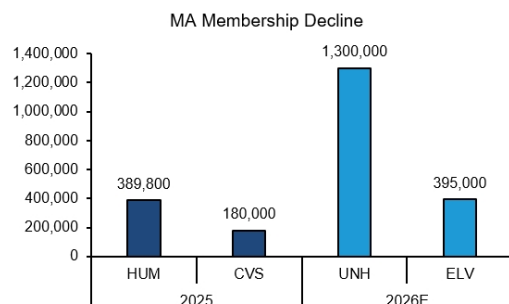
Source: NAIC, Bernstein analysis

CURRENT ENVIRONMENT AND OUTLOOK

Subsequently, the industry has seen a withdrawal of competition. HUM and CVS reduced MA membership in 2025 followed by UNH and ELV announcing reductions for 2026. Competitors have also exited including CI, MOH, and several Blue Crosses and not-for-profit organizations.

EXHIBIT 22: Withdrawal of competition in MA

Big 4 MA membership drops



MA Exits

- Cigna
- Molina (exiting MAPD)
- Premera Blue Cross (exited WA)
- BCBS of Kansas City
- BCBS Vermont (exiting in 2026)
- Ucare (MN/WI)

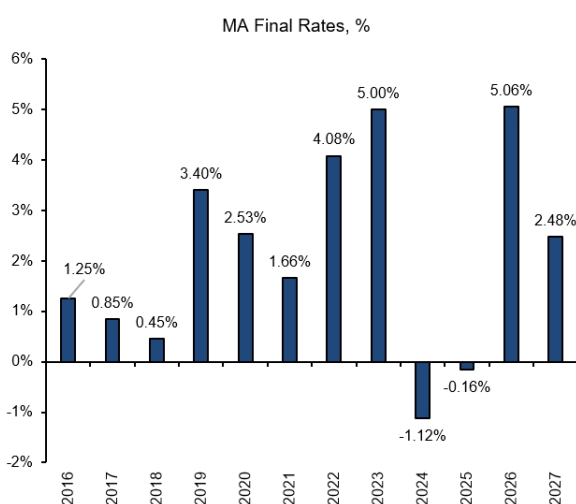
Source: Company reports, Bernstein analysis

In addition, the industry has seen a normalization in rates and utilization. 2026 saw a 5.06% rate increase, and the 2.48% rate increase for 2027 aligns with our view of normal long-term rates. The 2.48% rate in 2027 includes a reform of unlinked chart reviews, which lowered the excess risk coding for 2027 by another 1.5%. The 2027 rate identified, but did not adjust, another 2+% of potential adjustments to risk coding by deferring changes to the base year. The key takeaway is it appears there is only 2.5% or less of overcoding remaining in MA (vs 10% in 2023), which reduces rate risk for future years as well.

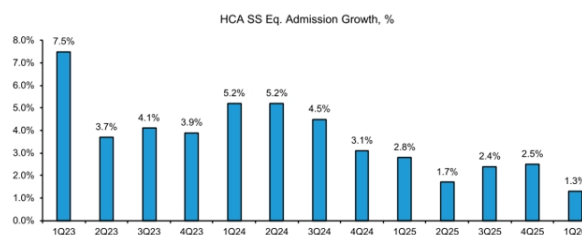
We are seeing the normalization of utilization patterns in medical volumes back to pre-COVID levels. For MCOs this is favorable relative to recent experience, and they are largely pricing based on the recent past experience in medical cost trends. For hospitals, this is a volume headwind, which many such as HCA, have incorporated into their guidance. We believe a return to more traditional levels of capacity growth (payroll growth) is a major element of this volume normalization.

EXHIBIT 23: MA rates normalize in 26, 27

MA Final rates

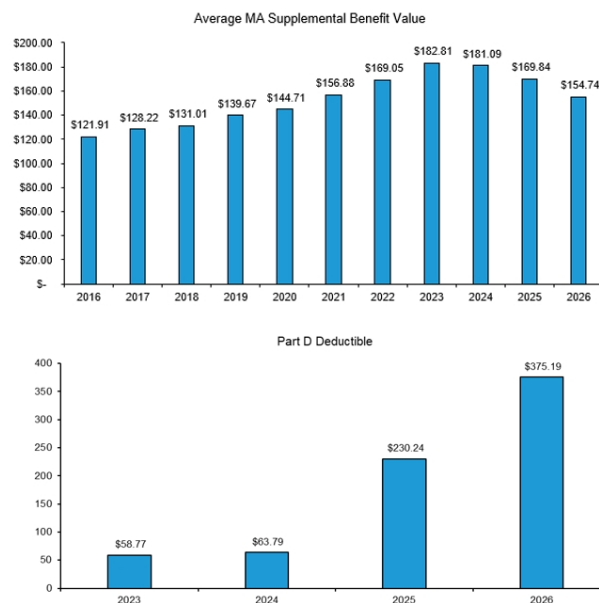
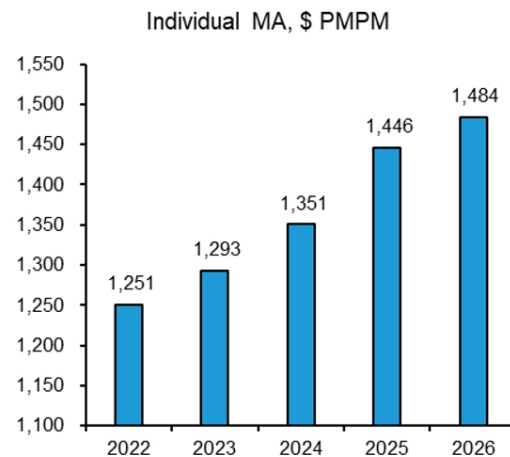


Utilization appears to be normalizing



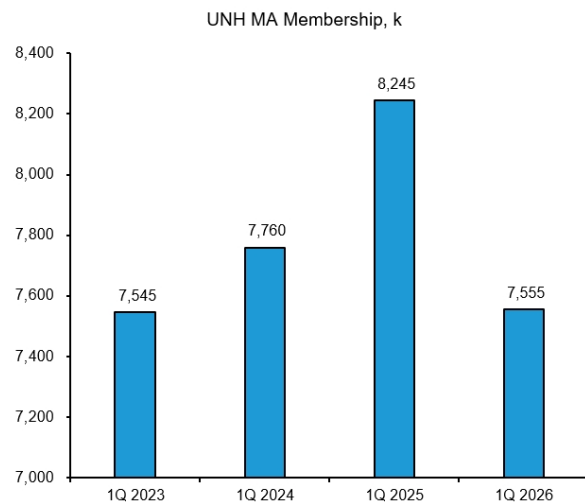
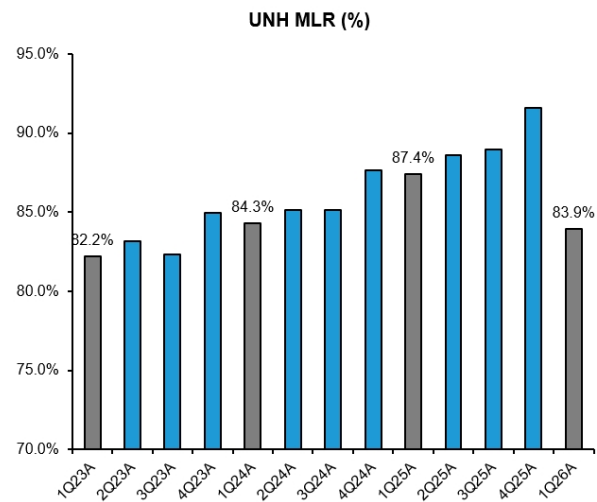
Source: CMS, Company report, Bernstein analysis

Benefit cuts are another mechanism to improve margins. Supplemental benefits peaked in 2023 and have come down in 2024, 2025, and 2026. Increasing deductibles on the pharmacy side has contributed to higher out-of-pocket expenses for members. Premiums have increased with the introduction of the Inflation Reduction Act creating additional MCO exposure to pharmacy costs. This trend can be observed in the figure below, which shows HUM's first quarter premiums.

EXHIBIT 24: **MA - benefits design****Benefit cuts including deductible and out of pocket increases****And premiums per policy have increased with the IRA and change to pharmacy benefit**

Source: CMS, MedPac, Milliman, HUM, Bernstein analysis

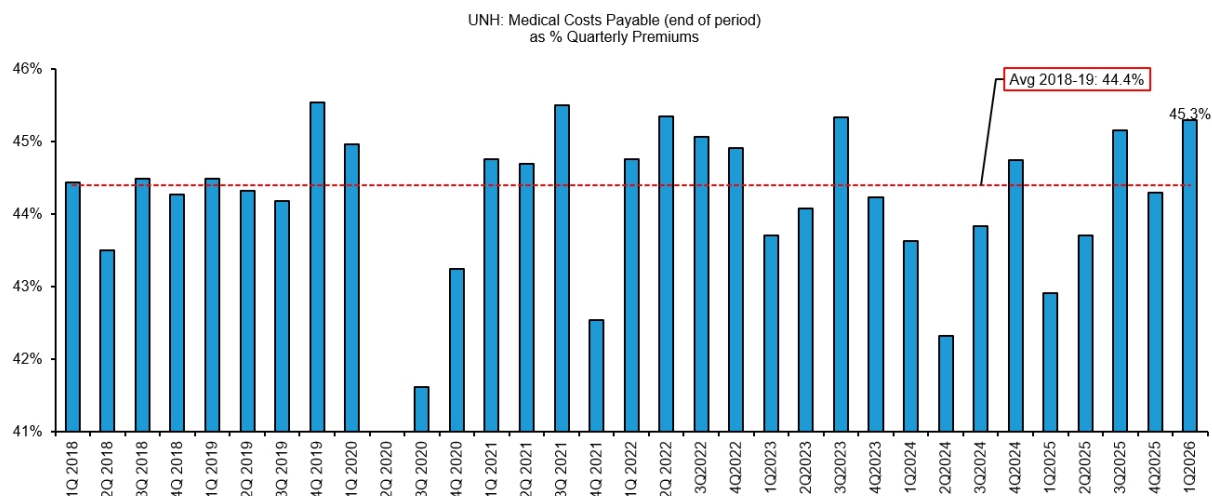
UNH has cut back on MA membership, and has shown significant normalized margin improvement in 1Q26. CVS saw a similar dynamic, improving 1Q26 MLR 200-300 bps. It's important to understand that MCOs have levers to impact margins during different policy environments. They can lose money in a strong rate environment and make margin improvements even under a low rate environment.

EXHIBIT 25: **MA plans pull back on membership growth****UNH MA Membership****Adjusted UNH MLRs**

(MLR for 2025 represent Bernstein's view on MLR adjusted for reserve timing)

Source: Company reports, Bernstein analysis

With the volatility of the past few years, strong reserving patterns have been observed. UNH's 1Q26 reserving is among the strongest it's been over the past 8 years. Resultingly, we look at this as a true turnaround in MLR, not just a temporary improvement due to reserving patterns.

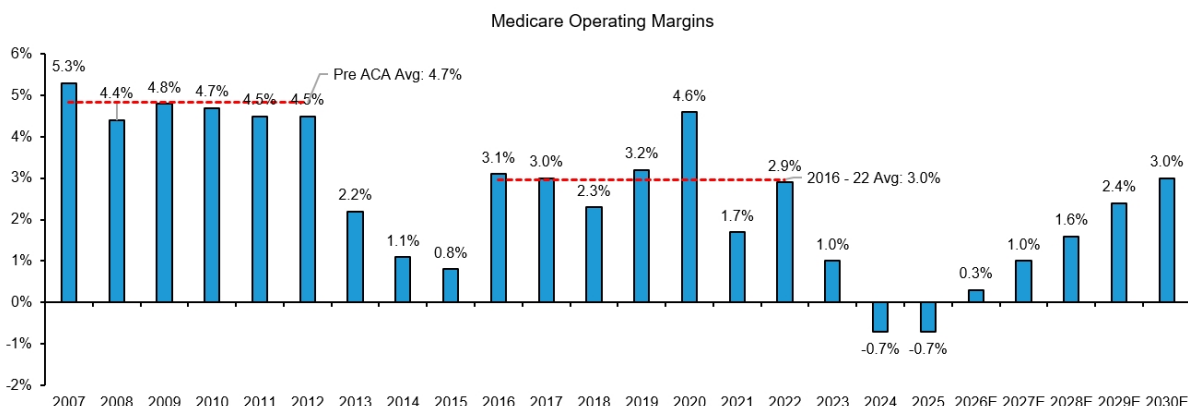
EXHIBIT 26: **Reserving****UNH – Medical reserve levels**

Source: Company reports, Bernstein analysis

We believe 2025 was a year of margin stabilization. We expect 100 bps of recovery at an industry level in 2026, with more upside possible. At a company level, we expect 50 bps improvement in 2027 and a continuation of that pattern through 2030. We expect increasing consolidation to top players with lower margin players exiting. This dynamic will make the margin recovery look slightly more dramatic than it actually is.

EXHIBIT 27: Bernstein industry estimates on MA margin recovery

Expect company level MA margin improvements of 100 bps in 26, 50bps in 27 and beyond



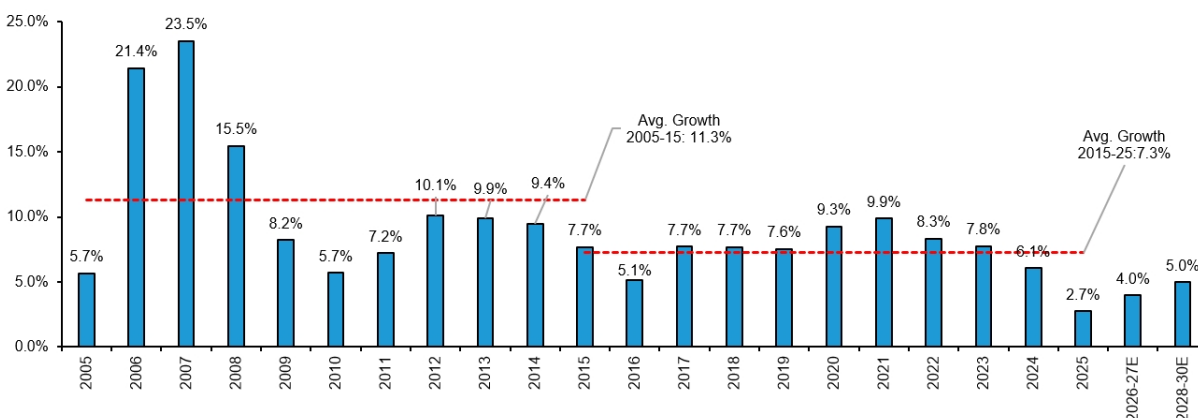
- Top end margins will likely be lower, as reduced competition causes fewer very low margin players

Source: NAIC, Bernstein analysis and estimates

We expect 4-5% long term MA growth on an industry level, including 4% in 2026 and 2027 and 5% in 2028-2030 as supplementary benefits rebound. This is a result of a larger base of membership, low rates leading to reduced pace of switching from FFS to MA, and slowing baby boomer retirements.

EXHIBIT 28: Bernstein estimates for industry growth

We expect 4-5% LT growth rate for MA membership



Source: CMS, Medicare Trustees Report, Bernstein analysis and estimates

FUTURE ENVIRONMENT AND KEY ISSUES

There remains approximately 2.5% of risk adjustment due to excess coding intensity that we believe will ultimately need to be removed. CMS has suggested this could be closer to ~50 bps based on its February analysis. We expect any adjustment to be phased in over several years, likely through ~100 bps annual reductions to effective growth rates. CMS is also evaluating potential changes to base years and home risk assessments. In addition, there is retrospective exposure from risk adjustment audits and potential settlements. ELV has set aside ~\$1B related to its ongoing suit around excess coding, while our prior estimates suggest UnitedHealth's exposure is in the ~\$2–4B range.

MedPAC has argued that, beyond coding intensity, favorable selection may represent an additional ~10 percentage points of excess margin. We do not agree with this view, a position that has recently been supported by both the WSJ and Paragon. Historically, MA may have benefited from favorable selection via avoidance of higher-acuity beneficiaries, resulting in ~10% selection bias. However, with MA penetration now ~55%, risk adjustment mechanisms that compensate for higher-acuity patients, and enrollment trends skewing toward lower-income populations, we view persistence of meaningful selection bias as unlikely.

Fiscal pressure remains an important backdrop. Medicare spending currently represents ~13% of the federal budget and is projected to rise to ~18% by 2032. With the Medicare trust fund expected to be depleted by 2033, funding gaps will need to be addressed either through increased government contributions or higher beneficiary premiums. As a result, we expect MA rate growth to lag underlying cost trend by ~100–150 bps on a sustained basis.

Over time, we expect CMS to recognize structurally lower costs in MA relative to FFS, which could support policies aimed at increasing MA penetration. The current administration has already begun evaluating mechanisms such as auto-enrollment, though no formal policy has been implemented to date.

EXHIBIT 29: Federal policy areas of focus

Risk adjustment	Fiscal pressures
- Remaining 2.5% (according to MedPAC) - Key topics – home risk assessments, changes to base years - RADV audits and prior overcoding exposure - Risk selection – MedPAC sees this as issue, we and others are skeptical	- Medicare represents 13% of federal budget - Will grow to 18% by 2032; Medicare trust fund runs out in 2033 - Likely to see MA rates 1 – 1.5% below medical cost trend over the long term, forcing MA plans to justify their value - Over time this will incent CMS to shift more enrollment to MA - This could lead to auto-enrollment

Source: CMS, MedPAC, and Bernstein analysis

We believe MCOs will become “utility-like.” In our view, the industry will consolidate around 4-5 large players, with MA growing to 100% penetration. As part of this, we expect the fragmented portion of the market, representing 40%, to move to the large scaled players. Long term, we expect the industry to have low margins and a higher leverage ratio as companies attempt to maximize ROE. We expect this shift to occur over the next ~20 years.

EXHIBIT 30: **Medicare Advantage plans as utilities****Government MCOs as utilities**

- We expect that government MCOs will become utility-like,
- We expect MA will grow to 100% penetration including administration for small portion of FFS business
- We expect Medicaid will outsource remaining spend with is 40+% of current spend
- We expect low margins (1.5% for Medicaid 2-3% for Medicare)
- We expect only 4-5 large nationally scaled players in each business

Source: Bernstein analysis

We are proponents that MA will move to a VBC model. Currently, about 1/3 of MA treatment is provided under a value-based care model. An alternative to pure VBC models are VBC-centric models, including Clover Health, that are disaggregating the principles of VBC and retaining them within an MA plan. These models are trying to engage physicians and enable physicians to accomplish the same goals as VBC but not through risk transfer, through bonus payments or paying a fee-for-service payment to perform additional services. The MA plan assists in coordinating the tasks that the physician would have to coordinate under a traditional VBC model. In our view, this makes the durability of MA higher, as it less likely to be purely disintermediated by VBC.

EXHIBIT 31: **VBC and MA plan centric enablement models**

Old FFS MA	Fully delegated VBC	Mixed MA controlled VBC / consumerism
• MA plan retains control • Focus on utilization management, prior authorizations • Rate increases and risk adjustment more important	• MA plan delegates risk to VBC company • Focused on driving VBC savings from more effective steerage to lower cost settings and proactive care to stabilize <u>chronics</u>	• MA plan retains risk • Manages medical costs through physician enablement and FFS payments • Coordinating various players in system • Consumer engagement is potential key enabler

Source: Bernstein analysis

The increased utilization of GLP-1s and other drug categories will inflate the cost of coverage, as demonstrated by pharmacy being a faster growing component of cost trend. GLP-1s also have the potential to increase life expectancy which could provide an additional 5-10% boost in Medicare enrollment.

EXHIBIT 32: **GLP1 disruptions****GLP1 implications**

- Over time, GLP1s are likely to be covered by Medicare
- This will present near term cost volatility, which is diminished the longer demonstration projects last
- The increased costs of coverage will inflate premiums
- Life expectancy increases are expected in the range of 0.5 – 2 years within 2 decades,
- Suggests a 5-10% increase in Medicare market enrollment in 1-2 decades

Source: Bernstein analysis

AI will have significant impacts moving forward. Artificial intelligence will lower operating expenses by improving payer operations and efficiency. On the consumer side, AI agents can facilitate and enable improved outcomes for patients.

EXHIBIT 33: **AI will be a major impact to healthcare including MA**

Category	Description	Value driver	Scope (\$Bn)	Opportunity (\$Bn)	Comments
Care Extension and Automation	Extending and automating care, adding volume per clinician	Adds volume per clinician	\$2,700	\$100+	Largest opportunity; ramping in niches near term, clinical automation will be longest term
Consumer Agent	Real time consumer agent which directs care navigation and spending	Improves cost effectiveness of clinical and drug spend	\$4,500	\$100+	Largest spend impacted, ramping today, medium term largest impact
Provider Efficiency	Automating care administration & operations	Automating admin, staffing and billing/insurance	\$500	\$80	Ramping in billing, staffing and admin present med term opportunity
Payer Automation and Efficiency	Automating insurance, claims and care management administration	Automating claims, call, enrollment and care management	\$200	\$40	AI to improve self-svc and electronic adjudication, med term
Precision Healthcare and Medicine	Personalizing care and treatment to reduce trial and error	Improves clinical effectiveness of spend	\$500	\$40	AI to enhance existing genomic aided efforts, med term

Source: Bernstein analysis

We believe vertical integration will remain an area of increasing policy scrutiny, although we view forced break-ups as a relatively low-probability outcome. A more realistic risk would be legislative efforts to effectively “stack” MLRs or cap margins across affiliated entities, which would reduce the economic rationale for maintaining integrated structures. In that scenario, traditional MCO and VBC businesses would likely be the first areas of focus, potentially leading to selective separation of these segments.

Importantly, we do not view this as inherently negative. In some cases, separation could unlock value or allow businesses to operate with greater focus. For example, we could see a scenario where UNH's VBC platform is able to grow more rapidly as a standalone entity.

Looking ahead, the 2028 election represents a key policy overhang. From a Democratic perspective, we expect the primary focus to center on expanding coverage back toward peak insured levels. The uninsured rate could drift toward ~9% by 2028, and reducing that back toward a more normalized ~5% level is likely to be a central policy priority, particularly against a backdrop of potential employment disruption from AI.

While “Medicare for All” continues to serve as a conceptual framework for broader system reform, political traction in recent cycles has been more closely tied to affordability and access. We expect these themes, rather than wholesale system overhaul, to dominate the policy debate. Although a Medicare for All–oriented candidate is likely to emerge, we believe coverage expansion will ultimately be the defining healthcare issue in the 2028 election cycle.

EXHIBIT 34: **Future policy issues**

Vertical integration

- Watching PBM and specialty pharmacy as “canary in the coal mine”
- Key topic is insurance companies and providers being jointly owned
- We think direct break-up is less likely
- We see MLR stacking as the issue which could cause a functional break-up of entities
 - Traditional MCO
 - PBM and cost containment businesses
 - Value based care

2028 election issues

Coverage is returning as an important issue – uninsurance is trending toward 9+%

- Restoration of peak ACA like coverage – Marketplace enhanced subsidies, broader Medicaid coverage, Medicaid expansion reform, provider taxes/state directed payments
- AI and employment uncertainty we believe will be a rising issue

Affordability remains the biggest issue

- Policies focused on affordability will be priority – drug pricing, out of pocket limits, hospital prices
- Medicare For All – less viable at 55% of Medicare in MA, was closer to 1/3 during Sen Sanders prior primary campaigns

Source: Bernstein analysis

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